



Littel Inc Emergency Hospital

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NURSING FLOWSHEET — VITAL SIGNS SURGICAL INTENSIVE / ORTHO POST-OP WARD · SHEET 1 OF 2

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
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DATE/TIME	BP	HR	RR	SPO2	TEMP °F	PAIN	POSITION	NEURO ✓	INIT
09/04 21:00	154/88	98	18	95%	99.0	10	HOB 30°	WNL	JM
09/05 01:00	142/94	97	16	97%	98.8	9	chair	WNL	RB
09/05 05:00	147/92	99	16	96%	98.8	10	amb w/ PT	WNL	TK
09/05 09:00	143/94	95	18	96%	98.9	10	supine	WNL	JM
09/05 13:00	142/91	94	16	97%	98.5	9	HOB 30°	WNL	CO
09/05 17:00	146/92	98	16	96%	98.2	8	chair	WNL	TK
09/05 21:00	148/88	99	16	96%	98.7	9	amb w/ PT	WNL	JM
09/06 01:00	142/93	94	18	97%	99.0	7	supine	WNL	PV
09/06 05:00	147/85	96	16	97%	98.8	8	HOB 30°	WNL	CO
09/06 09:00	144/89	96	16	97%	98.5	8	chair	WNL	RB
09/06 13:00	147/86	98	15	97%	98.9	8	amb w/ PT	WNL	TK
09/06 17:00	139/83	88	16	98%	98.5	7	supine	WNL	RB
09/06 21:00	140/90	88	18	96%	98.8	7	HOB 30°	WNL	TK
09/07 01:00	139/82	90	17	96%	98.5	5	chair	WNL	CO
09/07 05:00	134/89	90	16	96%	98.9	6	amb w/ PT	WNL	RB
09/07 09:00	143/85	88	17	97%	98.4	6	supine	WNL	CO
09/07 13:00	141/87	94	15	97%	98.4	5	HOB 30°	WNL	RB
09/07 17:00	138/85	94	15	97%	98.3	6	chair	WNL	PV

NURSING FLOWSHEET — VITAL SIGNS SURGICAL INTENSIVE / ORTHO POST-OP WARD · SHEET 2 OF 2

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
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DATE/TIME	BP	HR	RR	SPO2	TEMP °F	PAIN	POSITION	NEURO ✓	INIT
09/07 21:00	131/85	93	15	99%	98.0	5	HOB 30°	WNL	PV
09/08 01:00	141/82	87	16	97%	98.5	4	chair	WNL	TK
09/08 05:00	140/87	83	15	97%	98.0	5	amb w/ PT	WNL	TK
09/08 09:00	139/85	81	16	97%	98.5	4	supine	WNL	TK

Pain reassessed within 60 minutes of PRN analgesia per policy. Neuro checks q4h while concussion precautions active.

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 1 OF 2

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
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MEDICATION / ORDER	09/04/2023						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Oxycodone HCl 5mg Oral Tablet	—	PV	—	—	—	TK	CO
Ancef (Cefazolin) 1g IV	—	—	JM	—	—	—	—
Dilaudid (Hydromorphone) 1mg IV [PRN]	TK	—	—	JM	—	—	—
Ibuprofen 600mg Oral Tablet	—	—	JM	—	—	—	—

MEDICATION / ORDER	09/05/2023						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Oxycodone HCl 5mg Oral Tablet	—	PV	—	—	—	TK	CO
Ancef (Cefazolin) 1g IV	—	—	JM	—	—	—	—
Dilaudid (Hydromorphone) 1mg IV [PRN]	TK	—	—	JM	—	—	—
Ibuprofen 600mg Oral Tablet	—	—	JM	—	—	—	—

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 2 OF 2

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
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MEDICATION / ORDER	09/06/2023						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Oxycodone HCl 5mg Oral Tablet	—	RB	—	—	—	RB	CO
Ancef (Cefazolin) 1g IV	—	—	TK	—	—	—	—
Dilaudid (Hydromorphone) 1mg IV [PRN]	RB	—	—	—	—	—	—
Ibuprofen 600mg Oral Tablet	—	—	TK	—	—	—	—

MEDICATION / ORDER	09/07/2023						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Oxycodone HCl 5mg Oral Tablet	—	RB	—	—	—	RB	CO
Ancef (Cefazolin) 1g IV	—	—	TK	—	—	—	—
Dilaudid (Hydromorphone) 1mg IV [PRN]	RB	—	—	—	—	—	—
Ibuprofen 600mg Oral Tablet	—	—	TK	—	—	—	—

Initials: PV = P. Villanueva RN · CO = C. Okafor RN · JM = J. Marsh RN · RB = R. Bell RN · TK = T. Kovac RN · H = held (criteria not met) · — = not scheduled

**OPERATIVE REPORT: LEFT UPPER EXTREMITY RECONSTRUCTION**

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/07/2023

Operative Details

- **Date of Surgery:** 2023-09-07
- **Surgeon:** Dr. Myrtle Turner, MD, FACS
- **Assistant:** Dr. Charles Vance, MD (Resident)
- **Preoperative Diagnosis:**
 - Open compound mid-shaft left humerus fracture
 - Displaced spiral fracture of the left distal radius
- **Postoperative Diagnosis:** Same
- **Procedure Performed:**
 - Thorough irrigation and debridement of 2.5cm open wound, left upper arm.
 - Open reduction and internal fixation (ORIF) of left humerus fracture with an 8-hole dynamic compression plate and screws.
 - Open reduction and internal fixation (ORIF) of left distal radius fracture with a volar locking plate and screws.
 - Repair of disrupted local extensor tendon sheath, left wrist.

Surgical Narrative

The patient was brought to the operating room and general endotracheal anesthesia was induced. The left upper extremity was prepped and draped in the standard sterile fashion. First, the open wound overlying the humerus mid-shaft was explored, irrigated with 3 liters of normal saline under pulse lavage, and non-viable muscle tissue was debrided. The humerus fracture fragments were identified, showing a high degree of spiral displacement. Using bone reduction forceps, the fragments were anatomic-ally aligned. An 8-hole dynamic compression plate was selected and secured across the fracture with cortical and cancellous screws. Rigid fixation was achieved.

Attention was then turned to the left forearm. A volar incision was made over the distal radius. The spiral distal radius fracture was exposed. The bone fragments were carefully aligned. A volar locking plate was positioned and stabilized with locking screws. Fluoroscopic imaging confirmed excellent anatomic alignment and hardware placement. The extensor tendon sheath was repaired with 4-0 Vicryl. The surgical wounds were closed in layers. Sterile dressing and a sugar-tong splint were applied.

**DISCHARGE SUMMARY**

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/08/2023

Clinical Course

Patient Dax Vance was admitted on 2023-09-04 following a severe MVA. Following orthopedic stabilization, the patient underwent successful left arm reconstruction on 2023-09-07. He tolerated the surgery well, and post-operative pain has been reasonably controlled on a combination of oral Oxycodone and Ibuprofen. Wound sites are clean, dry, and intact with no signs of infection. The distal neurovascular status of the left arm is improved, with radial pulses at 2+ and capillary refill at <2 seconds. Sensory deficit over radial nerve distribution is slowly resolving but remains slightly diminished. Patient's neck and back pain remain moderate, which are treated conservatively with muscle relaxants and soft collar as needed.

Discharge Instructions

- Activity: Strictly non-weight bearing on the left arm. Keep splint dry. No heavy lifting.
- Pain Control: Oxycodone 5mg (1-2 tablets every 6 hours PRN) for 14 days, then transition to Ibuprofen.
- Referrals: Follow up with Dr. Myrtle Turner in 2 weeks for staple removal. Prescribed physical therapy 3 times per week for 6 months once surgical incisions are fully healed, focusing on left shoulder, arm, wrist, and cervical spine mobilization.
- Warn of signs of infection or deep vein thrombosis (DVT).